Flash Comment — September 9, 2026

Biotechnology

Pharvaris N.V.

Management Provides Clarity on ALT Elevations and Expresses Confidence for Launch

Overweight
Price Target: $65.00

Ticker	PHVS
Upside/(Downside) to Target	**70.9%**
Price (09/09/2026)	**$38.04**
52 Week Range	$20.65 - 43.25
Market Cap (MM)	$2,589

Source: Company Data, Wells Fargo Securities estimates, and Factset.
NA = Not Available, NC = No Change, NE = No Estimate

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Our Call

At a group breakfast at the WF HC Conf, mgmt discussed data on LFT elevations: including hepatic fragility in HAE, LFT signals in recent approvals, and ALT risk from androgen withdrawal. We see no impact on uptake of deucrictibant XR.

Initial Thoughts

Patient phenotype consistent with hepatic signal. Two patients experienced ALT slightly higher than 5x ULN. One pt had a history of androgen use. Note that discontinuing androgens is a well known risk for elevated LFTs in HAE patients (see below). The pt continued w/ treatment, later enrolling in the CHAPTER-4 OLE. The other pt had fatty liver disease and stopped deucrictibant treatment but continued to be followed, with LFTs subsequently returning to normal.

Management cites historical LFT elevations in HAE trials. Takhzyro Phase 3 had one LFT >8× ULN and three >3× ULN. Orladeyo had one ALT >8× ULN with AST >3× ULN. Dawnzera directly targets the liver, in trials, two subjects discontinued: one >5× ULN for 30 days and one ALT <5× ULN. No modern HAE LTP has an FDA label requiring on-treatment LFT monitoring.

Danazol and withdrawal of it is hepatotoxic in HAE. Primarily used in ex-US markets, 20-30% of CHAPTER-3 participants had historical danazol exposure. Danazol has a black box warning for hepatic injury and is also associated with steatosis in HAE. Notably, discontinuation of danazol in HAE patients is now known to be associated with a transient hepatitis, as described in a case series describing a 3-10x elevation of ALT, generally resolving after 20 weeks of stopping therapy.

Management expresses confidence in launch. PHVS predicts 150-250 new US patients each year, and expects to be the default tx in both ODT and LTP due to efficacy and ease of use. PHVS also noted optimism in switching patients from Orladeyo and also from injectables (for those seeking to reduce tx burden w/o sacrificing efficacy). PHVS also expects synergies in onboarding physicians and practices once both deucrictibant IR and XR are approved.

All estimates/forecasts are as of 9/9/2026 unless otherwise stated. 9/9/2026 13:40:44EDT. Please see page 3 for rating definitions, important disclosures and required analyst certifications.
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Investment Thesis, Valuation and Risks

Pharvaris N.V. (PHVS)

Investment Thesis

Our Overweight rating on PHVS reflects our view that deucrictibant's strong on-demand efficacy can drive meaningful uptake, displacing injectable therapies and EKTERLY. We also expect Phase 3 prophylaxis data to replicate Phase 2 results, supporting broad market capture from existing oral prophylaxis therapies and high use in new starts.

Target Price Valuation for PHVS

Our $65 price target is based on a probability-adjusted DCF sum-of-the-parts analysis of deucrictibant IR and XR for HAE and AAE-C1INH, assuming a 10% discount rate and no terminal growth. A 90% PoS was chosen to account for uncertainty in liver signal, awaiting more data from the OLE.

Risks to Our Price Target and Rating for PHVS

Key risks include safety signals in the CHAPTER-4 OLE and/or the Phase 3 CREAATE study in the prophylaxis setting, failure to secure FDA approvals in either the on-demand or the prophylaxis setting, potential stickiness of patients to current HAE therapies, stronger-than-expected shift of patient preference toward injectable prophylaxis options with long dosing intervals (Q3M or Q6M) or one-and-done gene editing that are currently being developed by competitors.

Companies Mentioned in Report

Company Name	Ticker	Last Price (09/09/26)
Pharvaris N.V.	PHVS	$38.04

Source: Wells Fargo Securities LLC Estimates, FactSet

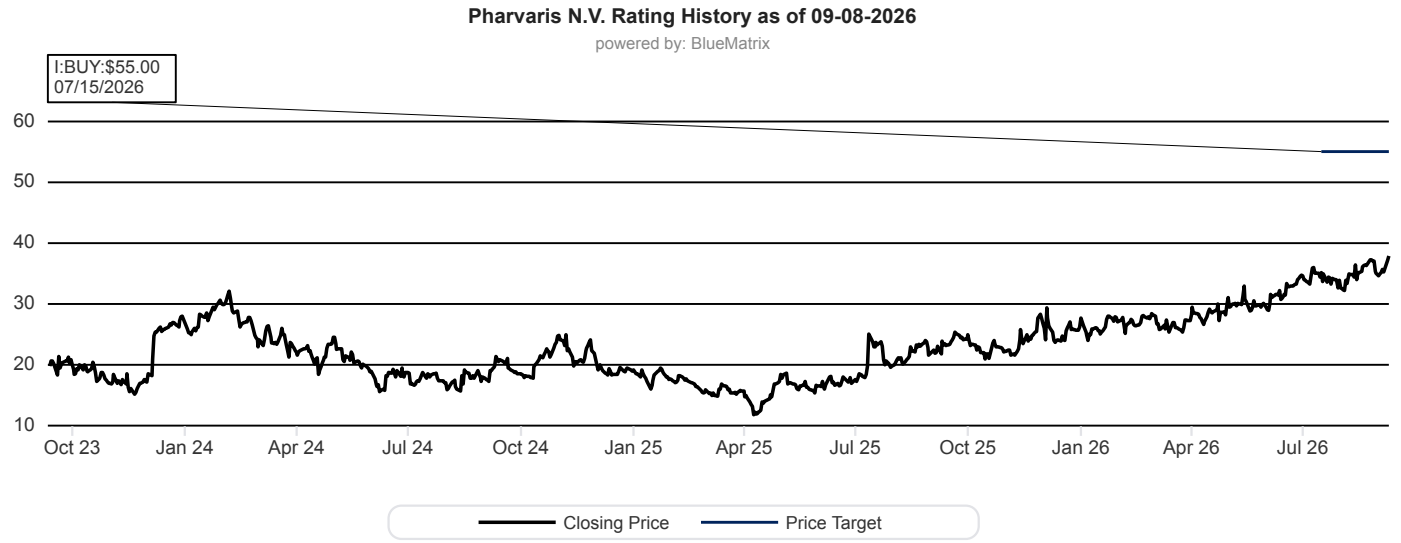
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